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MED 101: Introduction to Medicine - History Taking, Ethics and Professionalism

The human and professional foundations of medical training in Nigeria

FACULTY PRIMER

Faculty Primer

Faculty of Medicine
Community Medicine & Primary Health Care | 100L

Written for students, by students.

Practical notes, worked examples, practice questions and a revision plan.

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Independent study support | Not an official UNIZIK publication

START HERE

Why this pack exists

Medical school begins with learning how to listen. This pack introduces patient history taking, communication, confidentiality, consent and the Nigerian medical code of conduct, along with a practical study routine for a course load that punishes passive reading.

This is a student-written study companion, not a reproduced lecture handout or a leaked examination paper. Use it to organise the topics you are taught, practise the kind of thinking your course rewards, and walk into class with better questions.

What you will be able to do

- Take a structured patient history using a clear sequence.
- Apply the four principles of biomedical ethics to common clinical scenarios.
- Explain confidentiality, informed consent and professional boundaries.
- Build a sustainable study routine for high-volume medical content.

How to use this pack

1. Skim the whole pack once in 20 minutes so you know what is inside.
2. Work through one section at a time with your lecture note and recommended text beside you.
3. Attempt the practice questions before reading the answers. Mark what you missed.
4. Use the final checklist to plan your revision week, one tick per study session.

> **Free to download, always**
> This pack is free. There is no fee, no subscription and no payment step before download, and the full PDF is not watermarked or locked. Share the link with a coursemate instead of forwarding the file.

> **Confirm your official course outline**
> Course codes, credit units and outlines can differ by department and session. Treat this pack as supplementary support and confirm the current outline with your lecturer, course rep or departmental handbook before an assessment.

Structure of a history

A complete history is a method, not a conversation you improvise. Practice the sequence until it becomes habitual.

1. Bio-data and presenting complaint, in the patient's own words plus duration.
2. History of presenting complaint using the mnemonic for the relevant symptoms.
3. Past medical and surgical history, drug history, allergies.
4. Family, social and occupational history.
5. Review of systems.

6. Summarise back to the patient and confirm that you understood correctly.

> **Open then closed**
> Begin with open questions such as 'tell me what brought you in', then narrow with closed questions. Jumping straight to closed questions produces incomplete histories.

Ethics that will actually be tested

Ethical reasoning is a subject with principles, not a personality trait.

- Autonomy: respect the patient's right to decide, supported by adequate information.
- Beneficence: act in the patient's best interest.
- Non-maleficence: avoid avoidable harm.
- Justice: treat similar cases similarly and use scarce resources fairly.
- Confidentiality: information is shared only with those involved in care or as legally required.
- Informed consent requires capacity, adequate disclosure, understanding and voluntariness.

Professional behaviour and self-management

Studies show that unprofessional behaviour in training predicts later problems. Start the habits now.

- Arrive early, dressed appropriately, with the tools you need.
- Introduce yourself to patients and ask permission before examination.
- Do not discuss patients in corridors, lifts or on WhatsApp.
- Document honestly and promptly. A wrong note left uncorrected can harm a patient.
- Study with spaced repetition and active recall rather than rereading slides.
- Protect sleep, exercise and relationships - burnout damages clinical judgement.

> **The code of conduct**
> Read the current Nigerian Medical and Dental Council code rather than relying on hearsay. Know the sections on confidentiality, advertising and relationships with colleagues.

The clinical method

Medicine begins with a structured approach: history, examination, differential diagnosis, investigation, management. Each stage narrows the possibilities before the next, and most diagnoses are made from the history alone.

A differential is a working list ranked by likelihood and by danger. Rank by what is common, but never omit what is fatal - a rare diagnosis that kills matters more than a common one that resolves. This is the principle behind ruling out the worst first.

Management has four components: supportive care, specific treatment, prevention of complications, and communication with the patient. Students focus on the second and neglect the rest, which is why communication and consent are examined as clinical skills.

Systems and red flags

Chest pain is characterised by site, radiation, character, timing, aggravating and relieving factors and associated symptoms; red flags include pain at rest, diaphoresis and breathlessness. In respiratory disease, cough, sputum, haemoptysis, breathlessness and fever are the essentials.

The abdomen is examined in the order inspection, auscultation, percussion, palpation - auscultation before palpation because palpation alters bowel sounds. The neurological examination assesses conscious level, cranial nerves, tone, power, reflexes, sensation, coordination and gait.

Red flags require urgent action rather than routine follow-up: weight loss with night sweats, sudden severe headache, bleeding from any orifice, focal neurological deficit, and sepsis markers. Recognising them early is more valuable than memorising rare conditions.

> **The most useful question**
> Ask "what else could this be?" at every point. A single diagnosis reached too early is the commonest cause of clinical error.

Ethics, consent and professionalism

Consent must be informed, voluntary and given with capacity. Informed means the patient understands the diagnosis, the proposed intervention, the alternatives and the material risks.

Confidentiality has narrow exceptions: risk to others, statutory reporting, and the patient's own consent. Discussing a case in a public corridor is a breach, not a conversation.

Professional boundaries mean being honest about your level of training, never acting beyond competence, and reporting errors. Concealing mistakes harms patients more than the mistakes themselves.

WORKED EXAMPLES

Do it once, slowly

These are not model answers to copy. They show the reasoning a strong student would use, step by step, so you can repeat the process on a question you have not seen.

1. A differential for chest pain

A 22-year-old has sudden chest pain and breathlessness after a long bus journey.

1. List candidates: musculoskeletal, pneumonia, pneumothorax, pulmonary embolism, anxiety, cardiac.
2. Rank by likelihood: musculoskeletal and anxiety are common in the young.
3. Flag the dangerous: pulmonary embolism and pneumothorax.
4. Note the risk factor: prolonged immobility.
5. Specify immediate assessment: observations, saturation, chest examination.
6. State what would change the ranking: hypoxia, unilateral reduced breath sounds, tachycardia.

> **What to carry forward**
> Rank by likelihood, but never let a dangerous possibility drop off the list.

2. A focused history

A patient reports headache. What must you establish?

1. Onset: sudden severe onset is a red flag.
2. Character, site, radiation, severity.
3. Associated features: fever, neck stiffness, visual change, vomiting, neurological symptoms.
4. Aggravating and relieving factors, including analgesia use.
5. Past history: hypertension, trauma, migraine, medication.
6. Decide: urgent referral or routine assessment.

> **What to carry forward**
> Onset first. Sudden severe headache is a different problem from a gradual one.

EXAM TECHNIQUE

Questions with model answers

Written in the shape your course is usually examined, with the marking logic spelled out. Attempt each one under time before you read the outline.

Q1. Explain the components of informed consent.

15 marks

1. Capacity, voluntariness, information.
2. What information: diagnosis, treatment, alternatives, material risks.
3. Who may obtain it.
4. Exceptions: emergency and statutory.

Q2. Describe the structure of a clinical history.

15 marks

1. Presenting complaint and chronology.
2. History of the presenting complaint.
3. Past medical, drug, family and social history.
4. Review of systems.
5. Explain why structure reduces missed information.

REFERENCE

Glossary

Use these definitions in your own sentences; a copied definition rarely earns full marks.

Differential diagnosis. A ranked list of possible diagnoses for a presentation.

Red flag. A finding indicating serious disease and requiring urgent action.

Presenting complaint. The problem the patient reports as the reason for attendance.

Review of systems. A structured enquiry across body systems.

Capacity. The ability to understand and weigh information to decide.

Informed consent. Agreement given voluntarily with adequate understanding.

Confidentiality. The duty not to disclose patient information without justification.

Triage. Sorting patients by urgency to allocate care safely.

Sepsis. Life-threatening organ dysfunction from a dysregulated response to infection.

Iatrogenic. Caused by medical examination or treatment.

Prognosis. The likely course and outcome of a condition.

Professional boundary. The limit of role and competence within which a clinician may act.

Your four-week plan

Week 1 - Method

Reasoning.

- Take three simulated histories.
- Write a differential for each.
- Identify one red flag per case.

Week 2 - Examination

Systems.

- Practise cardiovascular examination in order.
- Practise abdominal examination in order.
- State normal findings aloud.

Week 3 - Ethics

Consent.

- Write a consent script.
- List confidentiality exceptions.
- Discuss one incident-reporting scenario.

Week 4 - Integration

Cases.

- Work three paper cases.
- Rank differentials by likelihood and danger.
- Present a case in three minutes.

Where to read more

Entry-level texts you can find in the university library or a departmental reading list. Confirm editions with your lecturer.

- A clinical skills textbook covering history and examination.
- A medical ethics text for undergraduates.
- Your faculty's professional conduct guidelines.

PRACTICE, NOT PREDICTION

Check your understanding

These questions are written for this pack to test the ideas above. They are not past examination questions and do not predict what your lecturer will set. Answer first, then compare.

1. What information must a patient understand before giving valid consent?

Answer: The nature of the procedure, its purpose, material risks and benefits, alternatives, and that they may refuse or withdraw.

2. Apply the principle of autonomy to a patient who refuses a blood transfusion.

Answer: A competent adult may refuse treatment even if refusal risks death. The team should explain consequences, explore reasons and document the informed refusal.

3. Why should open questions come before closed questions?

Answer: Open questions allow the patient to reveal concerns the clinician did not anticipate, improving diagnostic accuracy and rapport.

4. List three elements of a complete drug history.

Answer: Current medications including doses, adherence, allergies and adverse reactions, and over-the-counter or herbal use.

5. Give one way a student can breach confidentiality unintentionally.

Answer: Discussing a patient in a public space or sharing clinical images on a personal phone or social-media group.

Your 7-step action plan

Tick each step only when you have done it. A tick you cannot defend is worse than an empty box.

- [ ] Practise a full history on two friends and time yourself under ten minutes.
- [ ] Write definitions of the four principles of biomedical ethics with an example each.
- [ ] Read the current MDCN code of conduct and list five key duties.
- [ ] Draft your own consultation opening and closing phrases.
- [ ] Explain informed consent in a three-sentence answer from memory.
- [ ] Set up a spaced-repetition routine for anatomy and physiology.
- [ ] Shadow a clinical posting and write a one-page reflection.

Keep going

One pack will not carry a semester. Pair this with your lecture notes, tutorials, and the past questions your department actually releases. If a topic still feels unclear, take a specific question to your lecturer or course rep - "I do not understand the difference between X and Y" gets a better answer than "I do not understand this course".

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YOUR WORKSPACE

Notes, questions and things to ask

Use this page in class or during revision. Write questions you could not answer instead of copying slides.

Three ideas I can now explain without notes:

A question I still cannot answer:

For my lecturer or course rep:

Next revision action and date: